

MRCPsych Paper B

Topic Strategy, Concentration Guide & Time Distribution

Based on 13 recall papers spanning 2020–2025 | All sessions analysed

SECTION 1: OVERALL TOPIC DISTRIBUTION

The table below shows every major topic, its approximate share of the paper, recommended revision time, and the effort-to-reward ratio based on cross-session frequency analysis.

Priority	Topic	% of Exam Paper	% of Revision Time	Effort : Reward
● 1 — CRITICAL	Statistics & Research Methods	30–35%	30%	High effort / MASSIVE reward
● 2 — CRITICAL	Clinical Pharmacology (pregnancy, EPSEs, mood stabilisers, clozapine)	18–22%	20%	Medium effort / High reward
● 3 — HIGH	LD Syndromes & Genetics	8–10%	8%	Low effort once table learnt
● 4 — HIGH	Dementia (DLB, Alzheimer's, FTD, MCI)	7–9%	8%	Medium effort / High reward
● 5 — HIGH	Psychotherapy (CAT, DBT, Family Therapy)	7–8%	7%	Low effort / pure recall
● 6 — MEDIUM	Forensic Psychiatry	6–7%	6%	Low effort / specific facts
● 7 — MEDIUM	Child & Adolescent Psychiatry	5–7%	6%	Medium effort
● 8 — MEDIUM	Eating Disorders & Perinatal Psychiatry	4–5%	5%	Low effort
● 9 — EASY MARKS	Prevention (universal/selective/indicated)	2–3%	2%	Minimal effort / free marks
● 10 — EASY MARKS	ECT	2–3%	2%	Low effort / fixed facts
● 11 — EASY MARKS	Substance Misuse	2–3%	3%	Low effort
● 12 — LOWER PRIORITY	Old Age Psychiatry & Other Clinical	2–3%	3%	Medium effort

KEY INSIGHT: Statistics alone is 30–35% of the paper. Getting 75% of stats questions right is likely the difference between passing and failing — regardless of how well you know clinical content.

SECTION 2: STATISTICS — THE BATTLEGROUND (30% of revision time)

Statistics is the highest-volume topic by far. It appears as both MCQs and multi-question EMI stems. The same skills are tested every session — but with different graphs and numbers. Master the skill, not the specific question.

2.1 Within Statistics — Where to Spend Your Time

Stats Subtopic	Qs per Paper	Priority	What to Practise
Sensitivity, Specificity, PPV, NPV, LR+, LR–, post-test odds	4–6	● CRITICAL	Calculations from scratch — not just definitions
Statistical test selection (Chi-sq, t-test, ANOVA, Mann-Whitney etc.)	4–5	● CRITICAL	Memorise the decision table — 1 hour
Forest plot reading (significance, NNT, CI, effect size)	3–5	● CRITICAL	Practise with SPMM graphs
Study design (RCT, cohort, case-control, qualitative, nested)	3–4	● CRITICAL	Know when each is appropriate
Randomisation & bias (block, stratification, minimisation, ITT)	3–4	● HIGH	Definitions + what each controls
Health economics (QALY, ICER, cost types)	3–4	● HIGH	4 analysis types + 3 cost types
Validity & reliability (concurrent, predictive, convergent etc.)	2–3	● HIGH	8 terms — same scenarios recycled
Qualitative research (bracketing, triangulation, reflexivity)	2–3	● HIGH	6 terms — pure recall
Reporting standards (CONSORT, PRISMA, STARD, STROBE etc.)	1–2	● MEDIUM	One table — 8 guidelines
Confidence intervals, SEM, SD calculations	1–2	● MEDIUM	Formula: $\text{mean} \pm 1.96 \times \text{SE}$
NNT/NNH/ARR calculations	1–2	● MEDIUM	Practise from forest plots
Kaplan-Meier curves, hazard ratios	1–2	● MEDIUM	Interpretation only
Susser's criteria for causation	1	● LOW	Time order is the essential one

2.2 Critical Formulae — Know These Cold

Formula	Expression
Sensitivity	$TP \div (TP + FN) \rightarrow$ proportion of diseased who test positive
Specificity	$TN \div (TN + FP) \rightarrow$ proportion of non-diseased who test negative
PPV	$TP \div (TP + FP) \rightarrow$ chance patient has disease if test positive
NPV	$TN \div (TN + FN) \rightarrow$ chance patient does NOT have disease if test negative
LR+	$\text{Sensitivity} \div (1 - \text{Specificity})$
LR-	$(1 - \text{Sensitivity}) \div \text{Specificity}$
Pre-test odds	$\text{Pre-test probability} \div (1 - \text{Pre-test probability})$
Post-test odds	$\text{Pre-test odds} \times \text{LR+}$
NNT	$1 \div \text{ARR} \mid \text{ARR} = \text{CER} - \text{EER}$
NNH	$1 \div \text{ABI} \mid \text{ABI} = \text{EER} - \text{CER}$
SEM	$SD \div \sqrt{n}$
95% CI	$\text{Mean} \pm 1.96 \times \text{SEM}$ (approx. $\text{mean} \pm 2 \times \text{SEM}$)

2.3 Statistical Test Decision Table

Situation	Correct Test
2 groups, continuous normal data, DIFFERENT patients	Unpaired (independent) t-test
2 groups, continuous normal data, SAME patients (before/after)	Paired t-test
3+ groups, continuous normal data	One-way ANOVA
ANOVA but need to adjust for a covariate (e.g. age)	ANCOVA
2 groups, non-normal or ordinal data	Mann-Whitney U
3+ groups, non-normal or ordinal data	Kruskal-Wallis
Paired non-parametric	Wilcoxon signed-rank
2 categorical variables (2x2 table, all cells ≥ 5)	Chi-square
2 categorical variables (any expected cell < 5)	Fisher's exact test
Repeated categorical measures, SAME patients	McNemar test
Continuous outcome, multiple predictors	Multiple linear regression
Dichotomous (binary) outcome, multiple predictors	Logistic regression
2 continuous normal variables — correlation	Pearson correlation
2 ordinal or non-normal variables — correlation	Spearman correlation
Right-skewed data $\rightarrow$ convert to normal distribution	Log transformation
Survival / time-to-event analysis	Kaplan-Meier / Cox regression
Correlation controlling for a third variable	Partial correlation

2.4 Health Economics — 4 Types + 3 Cost Types

Type of Analysis	How Outcomes are Measured
Cost-utility (CUA)	QALYs → $ICER = (Cost A - Cost B) \div (QALY A - QALY B)$
Cost-effectiveness (CEA)	Single clinical measure (e.g. depression-free days, lives saved)
Cost-benefit (CBA)	ALL outcomes expressed in MONETARY terms
Cost-minimisation (CMA)	Treatments equivalent — choose cheapest option

Cost Type	Definition	Example
Direct	Directly tied to healthcare provision	Staff salary, medications, hospital transport
Indirect	Lost productivity of the patient	Sick leave, lost wages, carer time
Intangible	Cannot be easily quantified	Pain, suffering, stigma
Opportunity	Benefit foregone by using resource elsewhere	Nurse moved from pain clinic to ECT service
Incremental	Additional cost of one treatment over another	Cost of new drug minus cost of standard drug

SECTION 3: EASIEST MARKS — CONCENTRATE HERE FIRST

These topics take minimal time to learn but return guaranteed marks every session. Do these BEFORE going deep into statistics.

3.1 Prevention — 3–4 Questions Every Session, Same Scenarios

These exact 5 scenarios recycle in every paper. 1 hour of learning = 3–4 guaranteed marks.

Scenario	Correct Classification
At-risk mental state for psychosis + CBT	BOTH Primary Prevention AND Indicated Prevention (choose TWO)
School-wide anti-bullying programme for ALL students	BOTH Primary Prevention AND Universal Prevention
Low-income families mental health support	BOTH Primary Prevention AND Selective Prevention
Lithium to prevent further bipolar episodes	Tertiary Prevention
Alcohol price increase / restriction of sales	Universal Prevention
Screening programme for early detection	Secondary Prevention

3.2 LD Syndromes — 5–7 Questions Every Session, Same Syndromes

Memorise this table once. Test yourself daily for one week. After that it needs only occasional review.

Syndrome	Genetics	Must-Know Feature	Exam Clue
Fragile X	X-linked dominant; FMR1; CGG repeat	Gaze avoidance, cluttered/jocular speech, hyperactivity, anxiety	Gaze avoidance + jocular speech = Fragile X
Down syndrome	Trisomy 21 (95%); Robertsonian (5%, INHERITED)	Alzheimer's by 40; APOE4; passive/affable behaviour	Robertsonian = inherited equally from either parent
Lesch-Nyhan	X-linked recessive; HPRT gene; gout/hyperuricaemia	Severe self-biting (lips, fingers), spasticity, choreiform movements	Self-biting + choreiform + microcephaly in boy
PKU	Autosomal recessive; PAH deficiency	Mouse-like odour, hypopigmentation, microcephaly, seizures	Fair skin + musty/mouse odour
Smith-Magenis	Deletion 17p11.2	Self-HUGGING posturing, severe sleep disturbance, hoarse voice	Self-hugging = pathognomonic
Cri du chat	Deletion 5p	Cat-like cry, hypertelorism, round face, hypotonia, severe LD	High-pitched cat cry + hypertelorism
Rett syndrome	MECP2 gene; X-linked; girls ONLY	Normal development then regression at 6–18 months, hand-wringing, epilepsy	Girl + regression + hand wringing
Angelman	Maternal deletion 15q; genomic imprinting	Happy puppet; severe LD; paroxysmal laughter; no speech; epilepsy 86%	Happy puppet + loves water/music
Prader-Willi	Paternal deletion 15q; genomic imprinting	Hyperphagia, obesity, hypotonia; M:F = 1:1	Equal M:F; genomic imprinting
DiGeorge / 22q11.2	Chr 22 deletion; CATCH-22	Psychosis 20–30%; cardiac; cleft palate; hypocalcaemia	Psychosis + cardiac + hypocalcaemia
Friedreich's ataxia	Autosomal recessive; GAA repeat; chromosome 9	Gait ataxia, nystagmus, cardiac disease (MI cause of death)	Young man + ataxia + grandfather died from MI
Huntington's	Autosomal dominant; CAG >36; chromosome 4	Chorea; MRI = caudate atrophy; onset 30–50s	CAG >36 + MRI caudate atrophy

3.3 Neonatal Drug Effects — 4–5 Questions Every Session (EMI)

Neonatal Presentation	Maternal Drug
Jittery, high-pitched cry, poor feeding, sleep-wake disturbance	Opioids (heroin)
Overarousal, tachypnoea, tachycardia, agitation within hours of birth	Cocaine
Mild tremors, intermittent irritability, normal neonatal reflexes	Nicotine
Microcephaly, dysmorphic facies, low birth weight (~2 kg), behavioural problems	Alcohol (FAS)
Cleft palate, cardiac malformations, developmental delay	Sodium valproate
Mild irritability, neonatal discontinuation syndrome, PPHN (rare)	SSRIs (esp. paroxetine late pregnancy)
Floppy baby, cyanosis, bradycardia	Lithium

3.4 ECT — 3–4 Questions Every Session, Fixed Facts

ECT Fact	Answer
Most commonly affected memory	Retrograde (explicit/episodic)
Absolute contraindication	Phaeochromocytoma ONLY
Bilateral vs Unilateral	Bilateral = more effective; Unilateral = fewer cognitive SEs
Best predictor of response	Psychotic depression
Remission rate in TRD	60–80% (up to 90% in psychotic depression)
Relapse without maintenance	>80% within 6 months
ECT vs rTMS	ECT = more effective; rTMS = more targeted/specific
ECT + lithium	×12 risk of delirium — avoid if possible
Drug reducing seizure duration	Benzodiazepines; anticonvulsants — avoid/reduce dose
Drug increasing seizure duration	Clozapine; phenothiazines
Lofepramine/venlafaxine/moclobemide	Reduce dose or stop BEFORE ECT
NICE indications	Severe/life-threatening depression; TRD; severe mania; catatonia

3.5 Psychotherapy Key Terms — 5–6 Questions Every Session

Therapy	Must-Know Terms
CAT	Reformulation → Recognition → Revision (3 Rs). Initial: exploration + reformulation letter. Ending: goodbye letters. Key technique: sequential diagrammatic reformulation. Traps, dilemmas, snags.

Therapy	Must-Know Terms
DBT	Developed by Linehan (1991) for BPD. Phone calls = core technique. Pre-treatment = motivation + commitment. Hierarchy: self-harm → therapy-interfering → quality of life. Group + individual.
Systemic (Milan)	Re-framing + hypothesising. Non-expert stance. Circular/interventive questioning. Neutrality. Positive connotation.
Structural	Minuchin. Expert stance. Unspoken/unsaid rules. Hierarchy. Subsystems. Boundaries.
Strategic	Haley/Madanes. Paradoxical injunctions. Task/goal setting. Distorted hierarchies.
Narrative	Externalisation. Unique outcomes. Problem-saturated narratives. Therapist highlights exceptions.
Psychodynamic family	Past experiences. Object relations. Projective identification. Scapegoating. Holding environment.
Motivational Interviewing	DEARS: Developing discrepancy, Empathy, Avoid arguments, Rolling with resistance, Support self-efficacy. Amplifies cognitive dissonance.
IPT	RIIG: Role transitions, Interpersonal deficits, Interpersonal disputes, Grief.

SECTION 4: FORENSIC — SPECIFIC NUMBERS TO MEMORISE

Forensic questions test specific facts. Once learnt, these are reliable marks. Learn the numbers, not just the concepts.

Forensic Fact	Figure
Suicide risk in prison vs general population (males)	3–8 times higher
Suicide risk in prison vs general population (females)	>10 times higher
ASPD in prison population (male)	~50%
Female prisoners with BPD	~20%
Functional psychosis in male remand prisoners	~17%
Homicides committed by people with schizophrenia	~6%
Homicides by people with any mental disorder history	~30%
1 in 20 violent crimes committed by people with severe mental illness	5%
Age of criminal responsibility (England & Wales)	10 years
Restricted patients reconvicted within 2 years of community placement	7%
Sexual offenders who commit further sexual offences over 5–10 years	10–20%
Shoplifters with significant mental disorder	~5%

Forensic Fact	Figure
Schizophrenia in shoplifters	Less common than personality disorder
Suicide within 1 year of self-harm	0.7% (1 in 100)
Lifetime suicide rate in EUPD (BPD)	~10%
Lifetime suicide rate in schizophrenia	~6%
Prison suicides occurring within 6 months of imprisonment	Majority
Remand vs sentenced — higher suicide risk	REMAND (higher risk)
Single cell occupancy in prison	Risk FACTOR (not protective)
Being married in prison (unusual)	RISK factor (opposite of community)

Legal Concept	Key Point
Fitness to plead (Pritchard)	Must understand charges; instruct solicitor; decide plea; follow proceedings; challenge jurors; give evidence. AMNESIA does NOT make unfit to plead.
Sane automatism	EXTERNAL cause (concussion, one-off hypoglycaemia, sleep terrors) → complete acquittal
Insane automatism	INTERNAL cause (epilepsy, sleepwalking, brain tumour, recurring diabetes) → not guilty by reason of insanity
Diminished responsibility	Applies to MURDER only — reduces to manslaughter
Mens rea	Guilty MIND — intention
Actus reus	Guilty ACT — the physical act
Culpability	Legally and morally responsible for action or inaction
Bournewood case	Deprivation of Liberty Safeguards (DoLS)
Tarasoff case	Duty to warn third parties of patient's violent intentions
Osman case	Police duty to warn potential victims
Pritchard criteria	Fitness to plead test
Gillick/Fraser	Treatment of minors without parental consent

SECTION 5: PHARMACOLOGY — HIGH-YIELD SPECIFICS

5.1 Drugs in Special Situations

Situation	Drug of Choice	Drug to AVOID
Post-MI depression	Sertraline; mirtazapine	Fluvoxamine; citalopram; TCAs

Situation	Drug of Choice	Drug to AVOID
Epilepsy + depression	Mirtazapine; citalopram/escitalopram	Bupropion; clomipramine; amitriptyline
Epilepsy + hyponatraemia (carbamazepine)	Mirtazapine (lowest hyponatraemia risk)	All SSRIs (high hyponatraemia risk)
Liver disease + alcohol withdrawal	Oxazepam (liver disease); lorazepam (if IM needed)	Diazepam; chlordiazepoxide
Renal impairment eGFR <25	Haloperidol; olanzapine	Amisulpride; sulpiride (renally cleared)
Tamoxifen + depression	Sertraline; citalopram; venlafaxine	Paroxetine; fluoxetine; duloxetine; bupropion
Pregnancy — mixed affective episode	Quetiapine (preferred antipsychotic; low placental passage)	Valproate (absolutely avoid in women <50)
Breastfeeding — antidepressant	Sertraline (preferred); paroxetine (low milk/plasma ratio)	Lithium (contraindicated); doxepin
Haemorrhagic stroke + depression	Sertraline or mirtazapine	Citalopram high dose; TCAs
Parkinson's psychosis (no dementia)	Quetiapine or clozapine	Risperidone; olanzapine (worsen Parkinsonism)
Parkinson's dementia + psychosis	Rivastigmine (best evidence)	Antipsychotics (neuroleptic sensitivity)
Clozapine not working (partial response)	Amisulpride augmentation (best evidence); aripiprazole	Do not stop clozapine — check compliance first
Antipsychotic weight gain	Metformin (drug of choice)	No medication replaces lifestyle modification first

5.2 Mood Stabiliser Key Facts

Fact	Detail
Lithium → hypercalcaemia	Lithium RAISES calcium (valproate, carbamazepine, haloperidol LOWER it)
Lithium → thyroid	Hypothyroidism (20% risk middle-aged women); also hyperthyroidism; hyperparathyroidism
Lithium near delivery	Suspend 24–48h before induction/C-section; measure level 12h after last dose; restart day 1 postnatal; re-check at 1 week
Lithium monitoring in pregnancy	4-weekly until 34 weeks; then weekly until 2 weeks postpartum
Valproate teratogenicity	~10% risk of any birth defect; 1–2% neural tube defects; cleft palate; developmental delay; autism. AVOID in women <50
BALANCE study	Lithium > valproate monotherapy. Li + valproate > valproate alone. Lithium is standard first-line for prophylaxis.

Fact	Detail
Bipolar depression treatment	1) Olanzapine + fluoxetine 2) Quetiapine 3) Olanzapine 4) Lamotrigine / Lurasidone
Valproate → pancreatitis	Epigastric pain radiating to back + vomiting = investigate immediately
Anticonvulsant discontinuation	Minimum 1 month gradual taper (as mood stabiliser)

5.3 EPSE Management — Know the First Step

EPSE	First Step	Drug Treatment
Akathisia	Reduce antipsychotic dose	Propranolol (first-line); mirtazapine/mianserin; cyproheptadine
Tardive Dyskinesia (TD)	STOP anticholinergic (procyclidine) FIRST	Then reduce dose; switch to clozapine/quetiapine; tetrabenazine (only UK-licensed)
Pseudo-parkinsonism	Reduce dose / switch to SGA	Anticholinergics (procyclidine, benztropine)
Acute Dystonia	Anticholinergic urgently	IM procyclidine or benztropine
Clozapine hypersalivation	No licensed treatment	Pirenzepine; diphenhydramine; propantheline

5.4 QTc — Quick Reference

- QTc >500 ms → reduce dose IMMEDIATELY + consider urgent cardiology referral
- QTc 480–499 ms → reduce dose, repeat ECG, consider referral
- High-risk combinations: risperidone + citalopram; citalopram + diphenhydramine; duloxetine + diphenhydramine
- Electrolyte causes of QTc prolongation: hypokalaemia, hypocalcaemia, hypomagnesaemia
- Safest antipsychotics for QTc: aripiprazole; olanzapine

SECTION 6: 6-WEEK REVISION PLAN

This plan is calibrated to the actual frequency of topics across all 13 analysed recall files. Do not deviate from the priority order — especially in the first two weeks.

Week	Daily Focus	Key Resources	Target
Week 1	Stats: diagnostic test formulae (Sn, Sp, PPV, NPV, LR+, LR-, post-test odds). Practise 20 calculation Qs per day. Statistical test selection table.	SPMM Fresh Questions stats sets; Mock 14 stats section	Confident with all diagnostic test calculations

Week	Daily Focus	Key Resources	Target
Week 2	Stats: forest plots, meta-analysis (I^2 , fixed/random effects), study design, ITT, randomisation types, bias types, health economics, validity types, qualitative terms, reporting standards.	SPMM mocks — stats EMI stems. Psych-mentor stats module.	Can answer any stats EMI without notes
Week 3	LD syndromes table (memorise + test daily). Drugs in pregnancy + neonatal effects. ECT (8 fixed facts). Prevention (5 fixed scenarios).	Report tables; SPMM LD module. Maudsley pregnancy chapter.	LD table reproducible from memory
Week 4	Dementia: DLB, Alzheimer's, FTD, MCI vs dementia. Pharmacology: EPSEs, clozapine, mood stabilisers, drug interactions, QTc.	Oxford Handbook; Maudsley prescribing guidelines	Confident with dementia differential + drug choices
Week 5	Psychotherapy (CAT, DBT, family therapy key terms). Forensic (statistics + legal cases). Child psychiatry (ADHD, OCD, depression NICE guidelines). Eating disorders (MEED, refeeding).	SPMM psychotherapy + forensic modules. NICE guidelines.	Psychotherapy terms and forensic numbers reproducible
Week 6	FULL MOCKS ONLY. Do SPMM Mock 14 + 2 additional mocks. Review every wrong answer. No new reading — consolidation only.	SPMM full mock papers. May 2025 recall topics as checklist.	Consistent >70% on mocks before exam

Daily Habit Throughout All 6 Weeks

- 30 minutes stats calculations every morning — do not skip this on any day
- Review LD syndrome table every evening for the first 2 weeks
- After Week 2: review one previous wrong answer set daily

SECTION 7: ADDITIONAL TOPICS FROM EXTRA RECALL FILES

The following items came from the additional 5 files (2020–2022 sessions) and are not in the main report. Add these to your notes.

7.1 Forensic Epidemiological Statistics (Handwritten MCQ Bank)

Fact	Figure
Postpartum blues	Up to 50% of new mothers
Postpartum depression	10–15% of new mothers

Fact	Figure
Postpartum psychosis	1–2 per 1000 deliveries
BPAD postpartum relapse risk	~50%
MCI annual conversion to Alzheimer's	9.6% / 5–15% per year
LBD average life expectancy from diagnosis	~6 years
Valproate risk of any birth defect	~10% (1 in 10)
Valproate risk of neural tube defects specifically	1–2%
Schizophrenia if BOTH parents have it	40–50%
Schizophrenia lifetime suicide risk	~6%
Anorexia monozygotic concordance rate	~55%
MDD → subsequent hypomania/mania	7–12%
OCPD prevalence	2–3%
ADHD UK prevalence	2–5%

7.2 Additional Facts from March 2022 Paper

- Hoover test → used to confirm functional/conversion leg weakness (organic disease has opposite Hoover pattern)
- Galbraith plot → assesses between-study heterogeneity (alternative to I^2 / funnel plot)
- Moclobemide → NICE-recommended for treatment-resistant social phobia
- Staccato (dysarthric) speech → cerebellar ataxia
- Progressive loss of memory for WORDS → semantic variant FTD (svPPA)
- Pirenzepine → antimuscarinic used for clozapine-induced hypersalivation
- Least discontinuation syndrome → Fluoxetine (longest half-life, self-tapering)
- Weak central coherence → seen in BOTH ASD and ADHD
- Avoidant PD vs Social Phobia → avoidant PD is pervasive across ALL situations; fear of criticism/rejection is core
- Schizophrenia — father's socioeconomic class = same as general population (supports social drift theory, not social causation as primary explanation)

7.3 Additional Facts from Earlier Sessions (2020)

- Flight into health → patient stops therapy saying they feel better — a form of resistance/defence mechanism
- Manic defence → feeling grandiose or euphoric at a funeral (acting opposite to expected grief)
- CBCL (Child Behaviour Checklist) → assessment tool for ADHD in children
- ADOS → assessment tool for diagnosing autism
- WISC → cognitive assessment in children (premorbid intelligence)
- Wernicke's most common presenting sign → ophthalmoplegia alone (full triad is rare in practice)
- Mosaicism in Down's syndrome → higher IQ than Trisomy 21
- Snowball sampling → participants recruit similar others; used for rare/hard-to-reach populations
- REM sleep disorder first-line treatment → Clonazepam

- Pathological lying (pseudologia fantastica) → matrix of truth and fiction; seen in personality disorder / forensic settings
- Transactional analysis → Parent, Adult, Child ego states; understanding behaviour through inter-personal transactions
- Turner's syndrome → short stature, webbed neck, hypothyroidism, coarctation of aorta

Summary: Where to Concentrate

1st priority: Statistics every single day — calculations, test selection, graph reading. This is 30–35% of the paper.

2nd priority: LD syndromes table + drugs in pregnancy + ECT facts. Low effort, guaranteed marks.

3rd priority: Psychotherapy terms + prevention scenarios. 1–2 hours total, same questions recycled every session.

4th priority: Pharmacology (drug choices in special situations) + dementia differentials.

5th priority: Forensic statistics + child psychiatry NICE guidelines.

Based on systematic analysis of 13 recall files spanning September 2020 – May 2025